



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نعمه ثابت مرسى ==

العنوان : المقطم

الحي / المدينة : المقطم

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 20-09-2026

رقم البطاقة : 26011072601726 رقم التليفون : 01147945054

العمر (تاريخ الميلاد) : 07-11-1960 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا أتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام أشعه مقطعيه علي البطن والصدر بالصبغه [11127]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: نعمه ثابت مرسي== National ID: 26011072601726	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest and abdomen
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 نقيبه
 ثالث مرسى
 ١٠ التفتيش - الاسرات
 المنظم - القاهرة
 ٢٠٢٠.١١.٠٧ ٢٦.٠١٧ ٢٦
 LN4617989



٢٠٢٥/٢ ٢٦.٠١٦.٠٧ ٢٦.٠١٧ ٢٦
 ربه منزل
 انش
 المنطقة سارية حتى ٢٠٢٧/٢
 رتبة
 مستند







Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : نعيمة ثابت مرسى حسين

Sex:

3- Medical number:
Female

83200758

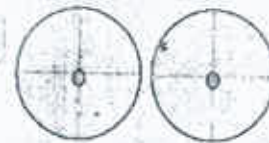
Old Med :

Old Med :

1197-2024

2-ID number:

Patient Age : 54 Y



Family History

Family History

Phone

Phone

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co-morbidity

Co-morbidity

Menopausal
status

Menopausal Status
WITH/WITHOUT

Initial STAGE

Initial STAGE

24/10/2024

Clinical data

Breast Imaging :

3/2024:rt breast suspicious looking lesion retroareolar 2.7 x2 cm
reaching nipple BIRADS 5 , rta xillary LN largest 1 x0.5 cm

Mets Workup :

ct cap 16/5/2024: rt breast UOQ mass 2.7 x2 cm , rt xillary
level I,II (Interpectoral) pathological LNs largest 2.2 x1.8
bone scan 6/2024:free

BX IDC BX COMMENT	ER 8/8 ER COMMENT	PR 5/8 PR COMMENT	Her 2 Neu 1 Her 2 Neu COMMENT	SISH SISH COMMENT	Ki67 12
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T 2	N 1	M 0
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Sonumammography

PATIENT HISTORY

RT MRM :IDC +DCIS PT2 MULTIFOCAL (3CM)
LN 9/12
free margins

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

- 1- ADJUVANT CHEMOTHERAPY ACX4 TAXOL X12
- 2- PORT
- 3- ADJUVANT AI(CONSIDER EXTENDED ADJUVANT
- 4- ZOMETA/6MONTHS

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Women's Health Outreach Program

Name: Naema Thabet

Date: 17-3-2024

Bilateral Full field Digital Mammography

Clinical summary:

Technique

Mediolateral oblique and CC views

High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously Dense mammary parenchyma (ACR b)
- A retroareolar density is seen , with nipple retraction
- Ultrasound :
A retroareolar lesion seen at the right breast seen at 12 o'clock 2 CMFN with internal vascularity measuring about 2.7x2 cm reaching the nipple .
No other cystic or solid masses seen.
- Normal skin/nipple/areolar complex.
- Right sided rounded suspicious looking axillary lymph nodes with loss fatty hilum, the largest seen measuring about 1x0.5 cm.

Left breast

- Heterogeneously Dense mammary parenchyma (ACR b)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound : No cystic or solid masses seen.
- Normal skin/nipple/areolar complex.
- Non specific axillary lymph nodes detected.

Impression :

Right breast suspicious looking lesion as described above (BIRADS 5), Biopsy was taken for histopathological correlation

Left breast normal BIRADS 1

Recommendation:.

Best regards,
Dr .Norhan osama.MD

Patient Name	الاستاذة / نعيمة ثابت مرسي	Request No.	13004980
Patient No.	260110/2601726	Request Date	18/03/2024
Path No.	1064/24	Verification Date	31/03/2024
Sex	Female	Financial Category	برنامج التأمين - بطاقة التغطية
Age	63 Y	Department	Out Patient
Referring Doc.		Registered by	marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Drug conjugates (ADCs) according to latest 2023 ASCO-CAP guidelines.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, < 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor cells.

Signature:

Prof. Dr. Lobna Omar,

Dr. Heba Hassan.

Verified by :

Printing Date : 31/04/2024 09:31:51

Page 2 of 2

Printed By : marwa mohamed youssef



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Verified by :
DR.Menna Nabil

Gross Examination:
• Procedure: Right MRM measured 23x19x5.5 cm with covering skin measured 22x14 cm.
• Tumor extent: An ill-defined firm grayish white mass.
• Size: 3 x2.5 x2.3 cm.
• Skin involvement: Retracted nipple & areola.
• Distance from margins: 4.5 cm away from deep margin.
• Lymph nodes number and size: Fibrofatty tissue piece measured 8x4.5x 2.5 cm entangling 12 L.Ns up to 2x2cm.

Microscopic Examination:
• Histological type: Multifocal invasive duct carcinoma
• Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
• Ductal carcinoma in situ: Focal DCIS (10%) (Comedo, cribriform and soli pattern).
• Skin involvement: Free nipple and areola.
• Lymph node involvement: Positive in (9/12) with Extracapsular extension.
• Resection Margins: Free deep margin.
• Micrometastatic findings: Positive.
• Additional pathologic findings: Moderate dysmoplasia.

Diagnosis:
• Right breast cancer Up-front , MRM specimen:
• Multifocal Invasive duct carcinoma GII with focal DCIS.
• Free nipple and areola.
• Lymph nodes were positive in (9/12) with Extracapsular extension.
• Free deep margin.

Clinical Data: Right Modified Radical Mastectomy, Up-Front
Nature Specimen: Right modified radical mastectomy specimen.

Pathology Report

Referred Name	26011072601726	Request No.	13001085
Path No.	3470/24	Verification Date	08/10/2024
Sex	Female	Financial Category	المرأة - فئة المرأة
Age	63 Y	Department	Out Patient
Referring Doc :		Registered by	marwa mohamed youss



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جدة رقم (4) - الدور الأول - مبنى رقم (987) - نيل كورنيش - ميسر القدامة - القاهرة، مصر

Printing By : Ali mostafa

Page 2 of 2

Printing Date : 08/10/2024 11:54:10

Verified by :
DR.Menna Nabil

Signature
Prof. Dr.Salma Tarek
Dr.Menna Nabil

* Stage: pT2, pN2a.

Pathology Report

Pathology Report	Referreing Doc :
Request No. : 13001085	Age : 63 Y
Request Date : 17/09/2024	Sex : Female
Verification Date : 08/10/2024	Path No. : 3470/24
Financial Category : فئة الجراحة - أورام دار السلام	Patient No. : 26011072601726
Department : Out Patient	Patient Name : / السيدة / مينا نabil
Registered by : marwa mohamed youss	



Verified by :
Dr. Duaa Mohamed

Comment: Although Her2 is Negative, Such cases (Score 1+) are predictive of response to Her2-Ab

- Ki67: Positive in 12% of tumor cells.
- Her2: (score 1+).
- PR: Positive (score 5/8).
- ER: Positive (score 8/8).
IPT:

Invasive duct carcinoma, Grade II.
Right retro-areolar mass, Tru-cut biopsy:
Conclusion:

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 70 % of tumor cells (score 8/8).
- Strong nuclear reaction in 9% of tumor cells (score 5/8).
- Her2: Faint incomplete membrane reaction in more than 10 % of tumor cells (score 1+).
- Ki67: 12% nuclear reaction of tumor cells.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Nature of Specimen: Tru-cut biopsy, collected on 17/3/2024, 10:10 am.

Clinical Data: Right retro-areolar mass.

IMMUNOHISTOCHEMISTRY REPORT

Patient Name	: /	Request No.	: 13004960
Patient No.	: 26011072601726	Request Date	: 18/03/2024
Path No.	: 1064/24	Verification Date	: 31/03/2024
Sex	: Female	Financial Category	: : فئة الجوال - أورام الثدي
Age	: 63 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
Registered by	: marwa mohamed youssef		





Verified by :
Dr.Duaa Mohamed

Signature
Prof. Dr. Lobna Omar.
Dr. Heba Hassan.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.
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drug conjugates (ADCs) according to latest 2023 ASCO-CAP guidelines.

IMMUNOHISTOCHEMISTRY REPORT

Patient Name	الاسماء / اسماء محمد	Request No.	13004960
Patient No.	26011072601726	Request Date	18/03/2024
Path No.	1064/24	Verification Date	31/03/2024
Sex	Female	Financial Category	المرضى - فئة الجراحة
Age	63 Y	Department	Out Patient
Referring Doc.		Registered by	marwa mohamed youssef

